MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 7 OF 28

MFT 707

Assessment and Diagnosis in Family Therapy

15 teaching weeks · 6 practicals · 3 figures

SEMESTER ONE

1.71 units · Prerequisite: MFT 701, 703

> “You will learn to gather multi-informant data, use standardised measures properly, formulate relationally, use diagnosis without being used by it, and communicate all of it to a family in words…”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 707 — Assessment and Diagnosis in Family Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 701, 703

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	**Assessment and Diagnosis in Family Therapy**	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. You will learn to gather multi-informant data, use standardised measures properly, formulate relationally, use diagnosis without being used by it, and communicate all of it to a family in words they can use. The course culminates in a full assessment report of publishable quality.

How to use this book

Assessment is the phase of therapy where the most harm and the most help can be done. Work through the weeks in order: the sequence matters, because each week's skill depends on the previous one. The final assignment is a complete assessment report — the same document type you will write for courts, schools and psychiatrists for the rest of your career.

1. Course description

Family assessment answers four questions: What is happening? How does it work? What maintains it? What would help? Diagnosis answers a fifth: Does this presentation meet criteria for a recognised disorder? Both matter, and they are not the same activity.

The course teaches the **multi-informant, multi-method** standard: self-report, collateral report, observation of interaction, standardised measures, records, and the family's own account — integrated in a formulation that is shared, tested and revised.

2. Learning outcomes

#	Outcome	Assessed
LO1	Plan and conduct a multi-informant family assessment	Observed assessment
LO2	Administer, score and interpret at least four standardised measures	Report
LO3	Use DSM-5/ICD-11 criteria appropriately, including their limits	Exam
LO4	Produce a 5-P formulation with relational hypotheses and a risk screen	Report
LO5	Identify medical, substance-related and cultural rule-outs	Exam
LO6	Give assessment feedback to a family in accessible language	Skills assessment
LO7	Write a report that is accurate, defensible and free of jargon	Report

3. Assessment

Component	Weight	Due
Measure administration and scoring (portfolio)	20%	Week 7
Observed assessment interview (30 min)	25%	Week 11

Component	Weight	Due
Full assessment report (3,000 words + appendices)	40%	Week 15
Examination	15%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Purposes of assessment: clinical, legal, institutional	Map the purposes in a case
2	The multi-informant principle; who to see and in what order	Design an assessment plan
3	History taking: development, family, medical, psychiatric, substance	Structured history interview
4	Interaction observation: what to look for and how to record	Live observation coding
5	Standardised measures I: relationship and family functioning	Score and interpret
6	Standardised measures II: symptoms, risk, children	Score and interpret
7	**Measure portfolio due**; psychometrics in practice	Peer review of portfolios
8	Diagnosis I: DSM-5 and ICD-11 — structure, use, limits	Criteria exercise
9	Diagnosis II: differential diagnosis and comorbidity	Case differentials
10	Medical, neurological and substance rule-outs	Referral-writing lab
11	**Observed assessment interview**	Assessment and feedback
12	Relational and systemic diagnosis: beyond the individual	Formulation workshop
13	Cultural formulation and bias in assessment	Cultural formulation exercise
14	Feedback, report-writing and shared decision-making	Report clinic
15	Integration: assessment that changes treatment	Case presentations

Week 1 — Purposes of assessment

Assessment is never neutral. Ask **who wants it and what they will do with it**.

Purpose	Who commissions	What they need
Clinical	The family	Understanding, direction, relief
Institutional	School, employer, agency	A defensible opinion and a recommendation
Legal	Court, solicitor, child protection	Facts, opinion within expertise, documentation
Research	Investigator	Standardised, comparable data
Self-understanding	The family	Narrative coherence

The trap of multiple purposes. A family may come for help and be sent by a court. The therapist must be explicit about which hat they are wearing, because the duties differ — a clinical assessment prioritises the client's interests; a forensic assessment prioritises accuracy for the court and is not confidential in the same way.

> **PRACTICAL**
> Take any referred case. List every party with an interest in the assessment and what each wants. Identify where the interests conflict.

Week 2 — The multi-informant principle

Rule. Never rely on one informant for a relational question. Mothers, fathers, children and teachers systematically disagree, and the disagreement is clinically informative.

A workable order of contact.

1. **Referrer** (5–10 minutes by phone): what is the concern, what has been tried, is there risk?
2. **Whole family** (60–90 minutes): joining, each person's view, interaction observation, contract.
3. **Subgroups** as indicated: parents alone, each child, each partner separately (always screen IPV separately).
4. **Collateral** with consent: school, GP, previous therapist.
5. **Measures** completed between sessions.
6. **Feedback session**: share the formulation, invite correction.

Who is missing? Fathers, paternal relatives, non-resident children, step-parents, grandparents who co-parent, and the person the family has decided not to discuss. Ask: "Who else is part of this family who is not here? Who would disagree with what has been said today?"

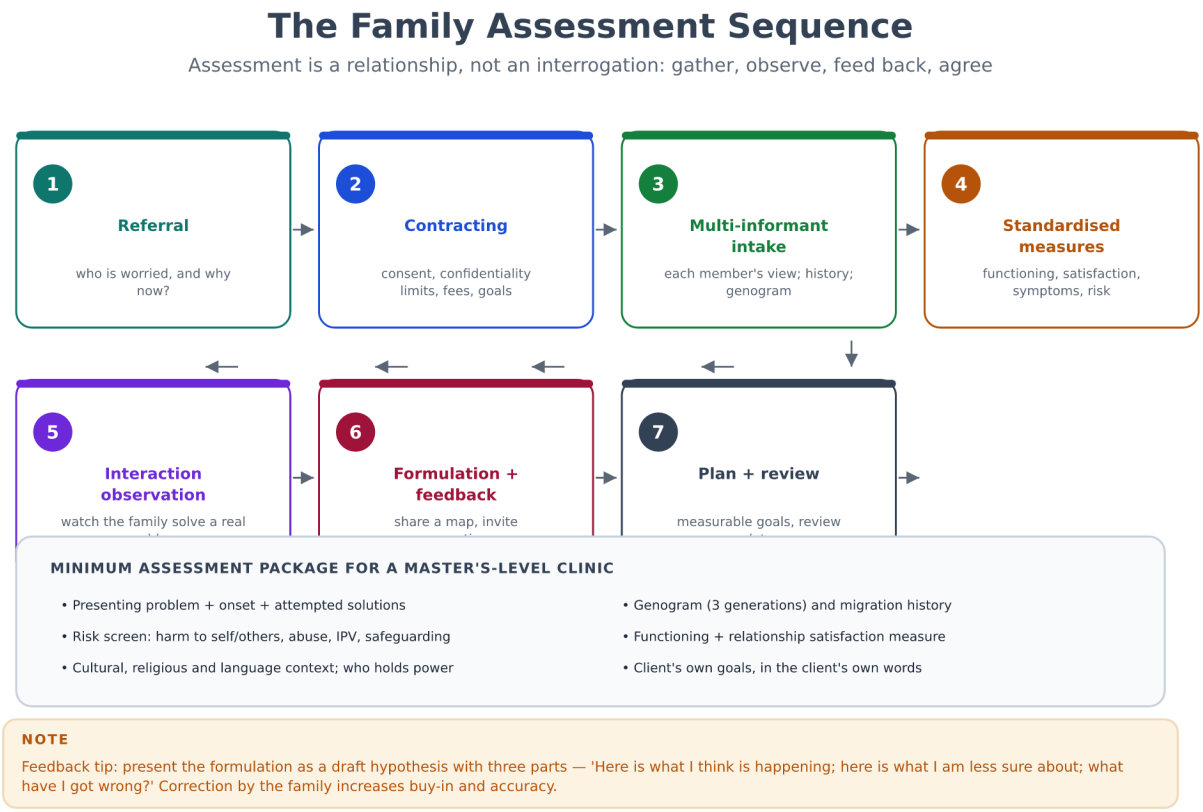


Figure 2.1 — The assessment sequence from referral to review

> **PRACTICAL**
> For a case you know, design the assessment plan: who, in what order, how long, and why. Defend the order.

Week 3 — History taking

A structured history prevents the two classic errors: never asking, and asking everything.

Domains (45 minutes is enough if you are organised).

Domain	Key questions
Presenting problem	Onset, course, severity, triggers, attempted solutions, previous treatment
Developmental	Pregnancy, birth, milestones, illnesses, separations, schooling
Family of origin	Genogram (3 generations), roles, losses, secrets, migration, violence, mental illness
Current family	Household composition, rules, routines, roles, money, division of labour, sexuality
Medical	Chronic illness, medication, sleep, head injury, neurological symptoms
Psychiatric	Previous diagnoses, hospitalisations, self-harm, suicide attempts, treatment response
Substance use	Alcohol, tobacco, cannabis, prescription, other; quantity, frequency, consequences

Domain	Key questions
Social	Work, school, faith community, friends, housing, legal issues, finances
Risk	Harm to self/others, abuse, neglect, IPV, safeguarding
Strengths	What has kept this family going? Who helps? What works?

The strengths column is not decoration. Families referred for assessment have usually been described in deficit terms for years. A careful strengths inventory changes the emotional tone of everything that follows and predicts engagement.

Week 4 — Interaction observation

You do not have to rely on what families say about themselves; you can watch them.

Four standard tasks.

1. **The problem discussion (10 minutes).** "Talk to each other about the problem for ten minutes. I will not interrupt." Observe: who speaks, who is interrupted, who looks at whom, who is silent, how it ends.
2. **The planning task (5 minutes).** "Agree on one change you will make this week." Observe: negotiation, hierarchy, coalition, flexibility.
3. **The positive reminiscence (5 minutes).** "Tell each other about a good day you had together." Observe: affection, repair capacity, humour.
4. **The child-focused task.** "Decide together how to respond to the next tantrum." Observe: who decides, whether the child is consulted, whether the parents support each other.

What to record (behavioural, not judgemental).

Record	Not
"Father interrupted mother 7 times in 10 minutes; she stopped speaking after minute 6."	"Father was dominant."
"Daughter looked at mother before answering each question."	"Daughter is enmeshed."
"Parents agreed on the plan in 90 seconds; the child's objection was not addressed."	"Weak parenting."

The five-minute structural map. Draw the seating, the direction of speech, and the coalitions you observed. It will be more useful than a page of adjectives.

Week 5 — Standardised measures I: relationship and family functioning

Why measure. To obtain a baseline, compare informants, track change, communicate with other professionals, and protect against your own impression management.

Core measures.

Measure	What it measures	Items	Notes
DAS-4 / RDAS	Couple satisfaction	4 / 14	Brief, widely used; cut-offs published
FACES-IV	Cohesion and flexibility	42	Psychometrics debated; use with caution
FAD (McMaster)	Family functioning	60	Six subscales; strong evidence base
Family APGAR	Perceived family support	5	Very brief; useful for screening
WAI-SR	Therapeutic alliance	12	Adapt for family members individually

Scoring discipline. Use the manual's scoring, not a photocopied table. Report raw and standardised scores, the norm group, and the cut-off used. State what a score does not mean: a high DAS score does not mean the couple is safe.

> **PRACTICAL**
> Score an RDAS from a completed (simulated) form: compute the total, compare with the published cut-off, and write three sentences you could say to the couple.

Week 6 — Standardised measures II: symptoms, risk and children

Symptom measures. PHQ-9 (depression; note item 9 is a suicide item and must be followed up), GAD-7 (anxiety), AUDIT (alcohol), DUDIT (drugs), PC-PTSD-5 or PCL-5 (trauma), YSQ or DERS (emotion regulation) as indicated.

Child measures. SDQ (emotional and behavioural, 4–17, parent/teacher/self), RCADS (anxiety and depression), CBCL (broadband; longer), Conners (attention). Always obtain multiple informants and report them separately.

Risk measures. Columbia Suicide Severity Rating Scale (C-SSRS) or a structured clinical interview; do not substitute a questionnaire for a conversation.

Interpretation rules.

1. Measure the same construct with the same instrument at follow-up.
2. Report informant discrepancies rather than averaging them away.
3. Treat screening positives as questions, not diagnoses.
4. Consider literacy, language and cultural fit before interpreting a score.

> **PRACTICAL**
> Administer the SDQ to a volunteer parent and the self-report to a peer playing an adolescent. Compute both. Compare. Write what the discrepancy might mean.

Week 7 — Psychometrics in practice



Figure 7.1 — Validity and reliability: four targets, three methods

Four questions to ask of any instrument.

1. **What is the norm group?** If it was normed on US undergraduates, a Nigerian clinical sample's score needs careful interpretation.
2. **What is the internal consistency?** α ≥ .70 is the usual minimum for research; ≥ .90 is desirable for individual clinical decisions.
3. **Has it been validated locally or in a similar population?** Search before you use.
4. **Is it responsive to change?** A measure that cannot detect improvement is useless for outcome monitoring.

Measurement invariance. The technical question "does this instrument measure the same thing across groups?" is clinically urgent. If a scale behaves differently for men and women, or across languages, then a difference in scores may reflect the instrument, not the people.

Week 8 — Diagnosis I: structure, use and limits

What DSM-5 and ICD-11 are. Classification systems that group patterns of signs and symptoms into categories, using criteria, thresholds and durations. They are reliable vocabularies with imperfect validity: they describe clusters, not known disease entities.

The five DSM-5 axes are gone (they were removed in DSM-5, replaced by a single axial diagnosis plus separate notation of psychosocial stressors, disability and contextual factors). Know the current structure even if you trained on the old one.

Using diagnosis well.

1. Diagnose to communicate, not to explain. "Generalised anxiety disorder" tells a colleague what to expect; it does not tell you why this woman, in this family, at this time.
2. Diagnose only what you can defend with criteria.
3. Record uncertainty explicitly: "provisional", "rule out", "not otherwise specified".
4. Consider the function of a diagnosis in the family: it can unlock care, or become an identity, or a weapon in a custody dispute.

Using diagnosis badly.

- **Premature closure:** the first label constrains all subsequent observation.
- **Diagnostic overshadowing:** the label hides a physical illness, a trauma, or a relational problem.
- **Category fallacy:** assuming that because a category exists, a distinct disease exists.
- **Cultural misreading:** interpreting culturally normative behaviour (spiritual experience, deference, grief expression) as pathology.

> **PRACTICAL**
> Take one case and write two formulations: one purely diagnostic, one purely systemic. Then write the paragraph that integrates them.

Week 9 — Differential diagnosis and comorbidity

The differential habit. For any presenting complaint, generate at least three alternatives before committing. Example: an adolescent's withdrawal.

Possibility	What would support it	What would rule it out
Major depression	Pervasive anhedonia, sleep/appetite change, 2+ weeks	No change in function; mood reactive
Social anxiety	Fear of scrutiny; avoidance specific to social settings	Comfortable with close friends
Trauma response	Intrusion, avoidance, hyperarousal after an event	No identifiable event
Substance use	Changed peer group, money, paraphernalia	Denial plus negative screen
Family crisis	Onset coincides with parental separation	No temporal link
Medical (e.g. anaemia, thyroid, sleep apnoea)	Fatigue, physical signs	Normal investigations

Comorbidity is the rule, not the exception. Depression and anxiety co-occur; substance use and trauma co-occur; child behaviour problems and parental mental illness co-occur. Prioritise: treat what is most dangerous, most disabling, or most treatable first, and treat the family context alongside.

Week 10 — Medical, neurological and substance rule-outs

Refer, do not diagnose. Your job is to notice and to route.

Presentation	Rule out	Route
First episode of psychosis	Organic causes, substance-induced	Urgent psychiatric referral
Sudden confusion in an elder	Delirium, infection, medication	Same-day medical review
Fatigue, weight change, low mood	Anaemia, thyroid, diabetes, sleep apnoea	GP with specific request
Memory decline	Dementia vs depression vs delirium	Cognitive assessment service
Headaches, seizures, personality change	Neurological	Neurology
Erectile dysfunction, low libido	Vascular, hormonal, medication	Medical review before psychologising
Unexplained somatic complaints	Physical illness, somatisation	GP plus psychiatric liaison

Write a referral that works. State what you observed, what you want excluded, why now, and who to contact. "Please assess and advise" produces a delay; "Please exclude anaemia and hypothyroidism in a 34-year-old woman with 3 months of fatigue and low mood, as I am considering a diagnosis of depression and would like to rule out physical causes before starting therapy" produces a response.

Week 11 — Observed assessment interview

Thirty minutes, assessed: contract and confidentiality, each person's view, interaction observation task, risk screen, strengths, and a closing summary with next steps. The family rates their experience of being assessed on three dimensions: heard, not judged, clear about what happens next.

Week 12 — Relational and systemic diagnosis

Individual diagnosis does not describe a relationship. Three ways of adding the relational layer:

1. Relational descriptors. Not a code, but a description: "a couple in a pursue–withdraw cycle with low repair capacity"; "a family with a cross-generational coalition and a disengaged father"; "a family at a stage transition with no renegotiation".

2. Global functioning of the relationship. Instruments such as the GARF (Global Assessment of Relational Functioning) rate the family or couple's functioning along a continuum, with anchors. Useful for outcome monitoring and for communicating severity.

3. The formulation as the relational diagnosis. The 5-P grid plus relational columns is the most clinically useful "diagnosis" a family therapist produces.

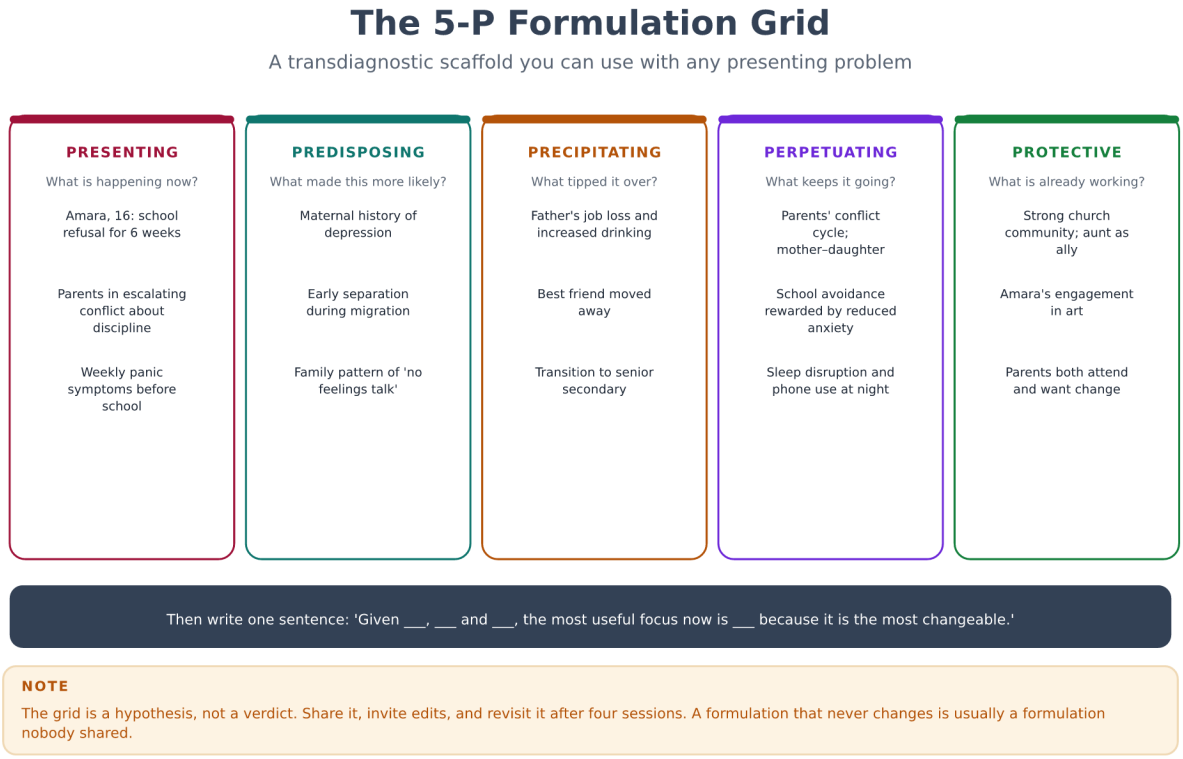


Figure 12.1 — The 5-P formulation grid applied to one family

Worked formulation (Adeyemi family, condensed).

> **PRESENTING.** Amara, 16, has not attended school for six weeks; panic symptoms on school mornings; escalating parental arguments about discipline. **Predisposing.** Maternal history of depression; paternal alcohol misuse; migration removed the extended family's daily support; a family rule that feelings are not discussed. **Precipitating.** Father's redundancy, a best friend's relocation, and the transition to senior secondary school, all within four months. **Perpetuating.** A pursue–withdraw cycle between the parents; a mother–daughter coalition that stabilises the marital distance; the relief Amara experiences when she stays home, which reinforces avoidance; disrupted sleep and night-time phone use. **Protective.** Both parents attend; a trusted aunt; church community; Amara's engagement with drawing; the family's capacity for humour. **Hypothesis (testable).** If the parents can complete one conflict conversation without Amara's involvement, her morning panic will reduce within three weeks. **Focus:** de-escalate the couple cycle while building a graded return-to-school plan that does not depend on Amara's compliance alone.

Week 13 — Cultural formulation and bias

The Cultural Formulation Interview (CFI) — a set of 16 questions from DSM-5 — gives structure to what many clinicians do informally:

1. What brings you here, in your own words?
2. What do you call this problem?
3. What do you think caused it?
4. How severe is it, and what worries you most?
5. What has helped, and what has not?
6. What kind of treatment do you expect?
7. Is there anything about your background or identity I should know?

Sources of bias in assessment.

Bias	Effect	Mitigation
Availability	Over-diagnosing what you saw last week	Use structured criteria
Confirmation	Seeking evidence for your first hypothesis	Actively seek disconfirmation
Cultural	Pathologising difference	Ask about meaning; consult
Gender	Reading assertion in women as pathology; in men as strength	Check your language
Class	Reading material hardship as neglect	Ask about money explicitly
Attribution	Blaming parents for adolescent behaviour	Include context and strengths

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Week 14 — Feedback and report writing

Feedback is an intervention. Structure it in four parts: what I heard, what I think is happening, what I am unsure about, what I suggest. Then stop talking and invite correction.

The report. Write for three readers: the family, the referring professional, and a future clinician who has never met you. Suggested structure:

1. Identifying information (minimal — no more than necessary)
2. Reason for referral (in the referrer's words)
3. Consent, confidentiality and who was seen
4. Sources of information
5. Presenting concerns, from each informant
6. History (developmental, family, medical, psychiatric, substance, social)
7. Observation of interaction
8. Measures administered, with scores and interpretation
9. Risk assessment and outcome

10. Formulation (5-P plus relational hypothesis)
11. Diagnosis, if any, with criteria met and differentials considered
12. Recommendations, with who will do what
13. Review date and signature

Style rules. No adjectives about people; no speculation without labelling; no jargon the family cannot read; no more than two pages for the family copy.

> **PRACTICAL**
> Write the family-facing summary (one page, no jargon) of your full report, and test it on a non-clinician.

Week 15 — Integration

Assessment is the moment a family learns what kind of problem they have. Get it right and therapy has a spine; get it wrong and every subsequent session is spent working on the wrong thing. The standard is not comprehensiveness; it is **sufficiency** — enough information to make a defensible formulation and a workable plan, gathered in a way that leaves the family feeling respected.

Glossary

Collateral information — data from third parties with consent. **Cultural formulation** — a structured account of the cultural context of a presentation. **Diagnostic overshadowing** — attributing a physical or relational problem to a psychiatric label. **Differential diagnosis** — the systematic consideration of alternative explanations. **GARF** — Global Assessment of Relational Functioning. **Measurement invariance** — whether an instrument behaves equivalently across groups. **Multi-informant assessment** — gathering data from several people who see the person in different contexts. **Rule-out** — a condition that must be excluded before a diagnosis is confirmed. **Screening** — a brief procedure identifying those who need fuller assessment.

Self-assessment

Multiple choice

1. The multi-informant principle exists because: (a) families lie (b) informants see different contexts and disagreement is informative (c) it is legally required (d) it fills the session

2. A high couple-satisfaction score means: (a) the relationship is safe (b) the relationship is satisfactory to the respondent, not that it is safe (c) the couple will stay together (d) no further assessment is needed

3. Diagnostic overshadowing refers to: (a) over-diagnosing (b) a label hiding another problem (c) comorbid diagnoses (d) cultural bias

4. The interaction task "agree on one change this week" is used to observe: (a) memory (b) negotiation, hierarchy and coalition (c) IQ (d) attachment classification

5. Which is the best referral request? (a) "Please assess and advise" (b) "Please see this patient" (c) "Please exclude anaemia and hypothyroidism in a 34-year-old woman with 3 months of fatigue" (d) "Please confirm my diagnosis"

6. The 5-P formulation stands for presenting, predisposing, precipitating, perpetuating and: (a) psychiatric (b) protective (c) prognostic (d) provisional

7. Screening instruments should be treated as: (a) diagnoses (b) questions requiring follow-up (c) evidence of malingering (d) definitive cut-offs

8. The CFI is: (a) a personality test (b) a cultural formulation interview (c) a child functioning index (d) a court report

9. The strengths inventory matters because it: (a) is required by codes (b) changes tone and predicts engagement (c) replaces diagnosis (d) shortens assessment

10. A treating therapist should generally decline to conduct: (a) an intake (b) a custody evaluation of their own client (c) a genogram (d) an outcome measure

Answer key

1 b · 2 b · 3 b · 4 b · 5 c · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. List the five parties who may have an interest in an assessment and what each wants.
2. Give three behavioural (not judgemental) observation statements.
3. What are the four parts of feedback to a family?
4. Name three biases in assessment and one mitigation for each.
5. Why must IPV screening be conducted separately from the partner?

Essay (choose one, 2,000 words)

1. "Diagnosis in family therapy is both necessary and dangerous." Discuss.
2. Using a disguised case, present a full assessment and defend your formulation against two alternatives.
3. Critically evaluate the claim that standardised family measures are culturally biased.

Further reading

- American Psychiatric Association. (2022). DSM-5-TR. APA Publishing.
- World Health Organization. (2019/2022). ICD-11.

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- Snyder, D. K., Heyman, R. E., & Haynes, S. N. (2005). Evidence-based approaches to assessing couple distress. Psychological Assessment, 17(3), 288–307.
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Next: MFT 708 — Cultural Competence in Therapy